

Case Document 6: Abdominal Tuberculosis with Acute Presentation

Patient Identification: Mrs. Amina Juma, 55-year-old Female. **Admission Date:** 18 May 2025. **History of Present Illness:** The patient presented with a two-month history of vague abdominal pain, significant weight loss, and low-grade evening fevers. She reported recent onset of abdominal swelling and altered bowel habits. She has a family history of respiratory disease but no previous personal history of chronic illness. **Hospital Course:** Initial ultrasound of the abdomen revealed mesenteric lymphadenopathy and mild ascites. An ascitic tap was performed, and PCR testing was positive for *Mycobacterium tuberculosis*. The patient was diagnosed with abdominal tuberculosis (abd tb). Stabilization was required for associated dehydration and electrolyte imbalance before transitioning to a long-term regimen. **Discharge Medications:**

- **Ceftriaxone:** 1g administered Intravenously (IV) twice daily (BD) for 3 days.
- **Septrin:** 0.96g administered Orally (Oral) once daily (OD) for 1 day.
- **Indication:** Initial management of abdominal TB and prevention of secondary peritonitis.

Case Document 7: Complex Chronic Exacerbation in a Geriatric Patient

Patient Identification: Mr. Robert Vance, 82-year-old Male. **Admission Date:** 02 June 2025. **History of Present Illness:** Mr. Vance was admitted via the emergency department with severe abdominal distension and worsening bilateral pitting edema in his lower limbs. His past medical history is significant for Type 2 Diabetes Mellitus (T2DM), Ischaemic Heart Disease (IHD), Chronic Obstructive Lung disease (COL/COPD), and chronic hypertension (HT). **Hospital Course:** The patient was found to have a severe exacerbation of his underlying respiratory condition complicated by fluid overload from congestive failure. He was managed with supplemental oxygen, nebulization, and diuresis. Due to his multiple comorbidities, a broad antibiotic coverage was initiated to treat a suspected secondary pulmonary infection. **Discharge Medications:**

- **Ofloxacin:** 0.4g administered Intravenously (IV) twice daily (BD) for 3 days.
- **Cefepime:** 1g administered Intravenously (IV) twice daily (BD) for 5 days.
- **Azithromycin:** 0.5g administered Orally (Oral) once daily (OD) for 3 days.
- **Indication:** Treatment for abdominal distension with leg swelling in a patient with chronic COL and IHD.

Case Document 8: Acute Gastroenteritis in a Patient with Miliary Tuberculosis

Patient Identification: Mr. David Miller, 48-year-old Male. **Admission Date:** 15 July 2025. **History of Present Illness:** A 48-year-old male with a known diagnosis of miliary tuberculosis presented with a 24-hour history of severe watery diarrhea and projectile vomiting. He was significantly dehydrated on arrival, with cold peripheries and a weak thready pulse. **Hospital Course:** The patient was diagnosed with acute gastroenteritis (acute ge) and septic shock. He received rapid fluid resuscitation with Normal Saline. Given his immunocompromised state due to underlying miliary TB, aggressive antibiotic therapy was initiated to prevent bacterial translocation and manage the acute infection. **Discharge Medications:**

- **Metronidazole:** 0.75g administered Intravenously (IV) three times daily (TDS) for 1 day.
- **Ceftriaxone:** 1g administered Intravenously (IV) twice daily (BD) for 1 day.

- **Co-amoxiclav:** 0.625g administered Orally (Oral) three times daily (TDS) for 5 days.
- **Indication:** Treatment for acute GE and stabilization in the context of miliary TB.

Case Document 9: End-Stage Renal Disease with Suspected Ischaemic Cardiomyopathy

Patient Identification: Mrs. Martha Stewart, 85-year-old Female. **Admission Date:** 21 August 2025.

History of Present Illness: The patient, an 85-year-old female, presented with progressive lethargy and orthopnea. She has an extensive clinical history of chronic cardiac failure (CCF), systemic hypertension, and iron deficiency anaemia (IDA). She is currently staged at CKD Stage 5 (End-Stage Renal Disease). **Clinical Assessment:** Physical examination showed jugular venous distension and bibasilar crackles. Echocardiography was ordered to investigate a potential diagnosis of Ischaemic Cardiomyopathy (?icm). Laboratory results confirmed severe renal impairment with elevated urea and creatinine.

Medication Summary:

- **Ceftriaxone:** 1g administered Intravenously (IV) twice daily (BD) for 7 days.
- **Indication:** Management of a suspected secondary chest infection complicating CCF and Stage 5 CKD.